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HAVEN PAEDIATRIC CENTRE

The Cultural Transformation Plan

Nineteen people, twelve weeks, one behaviour at a time

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01 Where we actually are

The town hall ran on 6 October and the team left oriented. Nineteen staff were emailed the same afternoon with the staff page, the pack and the deck. Three days on, the numbers are these.

HAVE SIGNED IN	THINGS SUBMITTED	TASKS COMPLETED
2 of 19	**3**	**0**
Sandra Okorie and Benedict Ijehon	one near miss, one pulse, one what-is-broken	of ten seeded, across three days

Two of nineteen is eleven per cent. It would be comfortable to call that a slow start. It is not: it is the number that tells us the design was wrong, and it is far more useful now than it would have been in January.

Two structural faults, and they are ours

A task cannot be ticked. Ten tasks were seeded and the morning email lists them, and there is no endpoint and no button anywhere that marks one done. We asked nineteen people to do things and gave them no way to say they had.

Nothing ever raises a flag. The table exists and is read, and nothing writes to it. So nothing was chased, and not doing a task cost exactly nothing.

Those two together explain the zero completely. No behavioural insight is needed for it and none should be claimed.

02 Why the other seventeen did not come back

Behaviour needs three things present at once: the capability to act, the opportunity to act, and the motivation to. Where a behaviour is absent, at least one is missing, and the useful question is which. Taking them in turn against what we know.

	Where Haven stands
Capability	Mostly fine. They have phones, they received the email, and the sign-in is a six-digit code with no password. The gap is narrow: nobody has been shown the page by a person, and watching somebody do a thing once is worth more than any instruction we write.
Opportunity	Weaker than we thought. The code goes by email, and most of the nineteen are on personal addresses they read at home rather than on shift. Text messaging is built and sits behind one unset key. After the first email nothing prompted anybody again.
Motivation	The real gap. Nothing on the page needed doing, nothing happened if they ignored it, and nothing visibly changed because anybody used it. We asked for behaviour and supplied no consequence in either direction.

The honest reading is that we built an invitation and called it a programme. A hospital of nineteen people, mid-shift, will not return to a page out of goodwill after the first week.

03 What we change, in three moves

Capability: make it possible, in person

Abigail and Dr Tanwa sit with each person individually, on shift, and watch them sign in once. Nineteen conversations of four minutes. This is not training and it is not announced as such; it is standing next to somebody while they do it once, which is the single highest-yield hour in this entire plan and the one most likely to be skipped as beneath us.

Opportunity: put it where they already are

The sign-in code moves to text message, which needs one environment key and is otherwise built. Text reaches a nurse on shift; personal email reaches her at home on Sunday.

Then the daily drip, covered in section 5. One message, one instruction, no digest.

Motivation: make something happen either way

Three things, in order of effect.

Close the loop out loud. The near miss already reported gets acted on this week, and what changed because of it is published to all nineteen with the reporter's permission or anonymously. One real example of somebody speaking and the hospital moving is worth more than every paragraph we have written about psychological safety.

Make the absence visible, gently. Flags start being raised. Not as a sanction: the first level is a question to the person, the second a quiet word from their line manager. The point is that not doing a thing stops being free.

Let people finish something. Completion is motivating and a list is not. Every person gets a short, visible run of things that can actually be ticked, and sees how many are left.

04 The sprints

Two weeks each, because that is the rhythm of the peer sessions and because a twelve-week programme with no internal joints becomes a twelve-week programme that slips once.

Each sprint targets **one behaviour**. Not three. The temptation is to run them in parallel and the result is that none of them sticks.

Sprint	Weeks	The one behaviour	Done means
1	13 to 24 Oct	Everybody signs in and completes one thing	15 of 19 signed in, each having ticked at least one task
2	27 Oct to 7 Nov	The daily task becomes routine	Half of all seeded daily tasks ticked across the week, two weeks running
3	10 to 21 Nov	Speaking up is normal	Six or more reports in the fortnight, and every one closed out visibly
4	24 Nov to 5 Dec	Protocols: acknowledge and study	Every person has acknowledged every protocol that applies to them
5	8 to 19 Dec	Protocols: tested	Eighty per cent passed on the protocols trained so far
6	5 to 16 Jan	It runs without us	A week where the cadence holds with no chasing from CFA

Sprint 6 falls after the season peak on purpose. Nothing new is introduced between 20 December and 4 January: the hospital is at its busiest, and a process introduced in that window will be blamed for everything that goes wrong in it.

05 The daily drip

One message a day, to one person, containing **one instruction**. Not a summary of outstanding items, which is a digest and gets archived.

When	07:00 Lagos, after the night shift and before the day one
What	The single most important thing waiting on that person, and nothing else
If nothing is waiting	Nothing is sent. This is what keeps the rest worth opening
Where it lands	Straight on the thing, never the front page
Friday	The exception: what changed this week because people spoke up

The existing morning email already computes the right list and already sends nothing when the list is empty. It needs narrowing from six items to one, and the Friday variant building.

06 Buddy pairs

Everybody is paired, and the pairings are published. Pairs meet for fifteen minutes a week, on shift rather than after it.

Four rules, each of them a way this normally fails.

Pair across shifts, so the pair is a line of communication the rota does not already provide. Pair across functions where sensible, because a nurse and a front desk colleague understanding each other's constraints fixes more friction than either being trained again. Never pair somebody with the person who writes their appraisal. And rotate every quarter, so the network ends up dense rather than becoming six fixed friendships.

The pair has one job at a time, tied to the current sprint, so it has a reason to meet. In sprint 1 that job is simply: have you both signed in.

07 Who does what

Project lead, CFA	The build, the sequencing, the daily drip, the measurement, and this plan. Accountable for whether the behaviour changes, not for whether features ship.
Abigail Oladejo	The chase. Who has not signed in, who has not ticked, who is presenting on Thursday. Schedules the peer sessions, records attendance, keeps the roster and the pairings current. The chase is the job most likely to be dropped and it is the one that makes the rest work.
Dr Tanwa Gbadebo	On site. The nineteen four-minute sit-downs. Clinical credibility with the nursing team, which neither of the others has. Chases the protocol bodies out of Dr Odedina and Dr Shakirah, and owns the operational tasks in her own right.
Dr Odedina and Dr Shakirah	Approve clinical content. Nothing generated reaches a nurse without one of them.

Escalation to Debo: anything commercial, anything involving the board, and anything touching the sensitivities.

08 How we will know

Four numbers, reported weekly, and the first is the only one that matters until it moves.

	Now	Sprint 1	End of plan
Signed in at least once	2 of 19	15	19
Used it in the last seven days	2	10	14
Daily tasks ticked, share of those seeded	0%	20%	60%
Reports in the fortnight	3	4	8 or more

The last one is the only measure here where a rise is unambiguously good and will look like bad news to anybody reading it cold. The board needs telling that before the first report, not after.

09 What would make this fail

Four things, named so they can be watched for.

We build more features instead of chasing nineteen people. The build is mostly done; the work now is human and it is duller.

The daily drip becomes a digest. The moment it carries six items it is archived unread, and the one that mattered goes with it.

A flag is used as a reprimand. One investigation that feels like a hunt costs a year of reports, and we will not get the reporting culture back inside this engagement.

We introduce something new in late December. The season is when the hospital is least able to absorb change and most likely to blame whatever is newest.

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